IP38-17: Implementation of a Multidisciplinary Quality Initiative to Mitigate and Reduce the Side Effects of Androgen Deprivation Therapy (ADT) in Prostate Cancer Patients Based on the Prostate Cancer 360 (PC360) Working Group Recommendations

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BACKGROUND AND RATIONALE

Androgen deprivation therapy (ADT) remains a cornerstone of prostate cancer treatment but is associated with substantial treatment-related morbidity. Despite extensive evidence describing cardiometabolic, skeletal, sexual, and psychological adverse effects, these complications are often under-recognized and inconsistently managed in clinical practice, reflecting the absence of practical, integrated care frameworks to guide comprehensive ADT management.

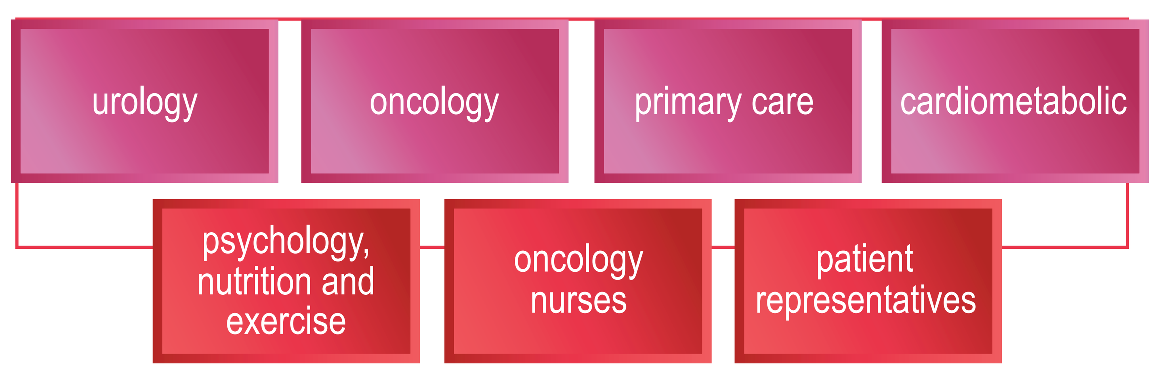
The Prostate Cancer 360 (PC360) Working Group convened a multidisciplinary panel to develop structured, consensus-driven recommendations aimed at supporting proactive monitoring, risk mitigation, and prevention of ADT-associated complications across the care continuum.

PC360 DEFINED

The PC360 Working Group convened 14 multidisciplinary experts in prostate cancer and ADT management to systematically identify gaps and challenges in the recognition and management of ADT-related adverse effects. Using a structured consensus process, the panel developed actionable, prescriber-focused recommendations. Panel composition included urology, medical oncology, cardiology, endocrinology, primary care, and survivorship specialists. PC360 is a structured multidisciplinary care framework designed to standardize monitoring and management of ADT-related toxicities across 5 core health domains.

Five core health domains were identified to inform overarching recommendations supporting the development of a Chronic Care Management (CCM) framework for patients receiving ADT. This model emphasizes coordinated management of chronic comorbid conditions through multidisciplinary care, in contrast to a Principal Care Management (PCM) approach that focuses primarily on specialist-led management of the prostate cancer itself.

Participating Disciplines in the PC360 Initiative


urology
oncology
primary care
cardiometabolic
psychology, nutrition and exercise
oncology nurses
patient representatives

Objective: To evaluate the feasibility and early clinical impact of implementing the PC360 multidisciplinary care model in routine urology practice.

STUDY DESIGN

Prospective quality improvement initiative evaluating implementation of PC360 recommendations within participating practices. Baseline characteristics and early clinical metrics were collected during the initial enrollment period. Patients receiving ADT were enrolled in a Chronic Care Management program with multidisciplinary monitoring across 5 health domains.

- Prospective, interventional, non-randomized quality improvement study
- Multicenter (academic + community)
- 300-500 patients expecting to remain on ADT >12 months
- 24-month project duration: assessments at baseline, 12 months, and 24 months
- Follow up for up to 36 months

PC360 CLINICAL PATHWAY

- Structured Clinical Workflows
- Standardized Monitoring
- PCP Engagement
- Education + Referrals
- Partner Engagement

PRIMARY ENDPOINT
- Pathway adherence

SECONDARY ENDPOINTS
- LDL-C, BP, A1C, eGFR
- BMD, vitamin
- IIEF-5
- PHQ-9, PHQ-2, GAD-7
- Activity adherence

EXPLORATORY ENDPOINTS
- Dental complications, ONJ
- Fractures
- MACE
- Safety signals

A1C, hemoglobin A1C; ADT, androgen deprivation therapy; BMD, bone mineral density; BP, blood pressure; DEXA, dual-energy X-ray absorptiometry; eGFR, estimated glomerular filtration rate; GAD-7, Generalized Anxiety Disorder scale 7-item; IIEF-5, International Index of Erectile Function 5-item; LDL-C, low-density lipoprotein cholesterol; MACE, major adverse cardiovascular event; ONJ, osteonecrosis of the jaw; PC360, Prostate Cancer 360; PCP, primary care physician; PHQ-9, Patient Health Questionnaire 9-item.

PC360 5 DOMAINS + RECOMMENDATIONS

High prevalence of uncontrolled cardiovascular risk factors in men starting ADT
- Baseline and ongoing risk assessment
- Early statin consideration
- Collaboration with primary care physicians (PCPs) and cardiology

Rapid bone loss and fracture risk associated with ADT
- DEXA scans, fracture risk assessment, vitamin D/calcium supplementation
- Antiresorptive therapy when indicated

Increased risk of depression, anxiety, emotional lability, and cognitive changes
- Routine screening (PHQ-2/9, GAD-2/7)
- Early referral pathways

ADT profoundly affects libido, erectile function, and intimacy
- Emphasize expectation-setting, partner involvement, and reframing sexual health beyond erectile function

ADT causes unfavorable body composition changes
- Strong emphasis on physical activity, nutrition, and behavior change as foundational interventions

NOTABLE INITIAL RESULTS

To date, 270 men have enrolled in the program. Preliminary implementation demonstrated successful integration of multidisciplinary care pathways, with high patient engagement and improved coordination across specialties. Early trends suggest improved monitoring of cardiometabolic risk factors and bone health, though longitudinal outcomes are ongoing.

15% are not under the care of a cardiologist

31% have not received a DEXA scan

32% are not taking vitamin D supplements

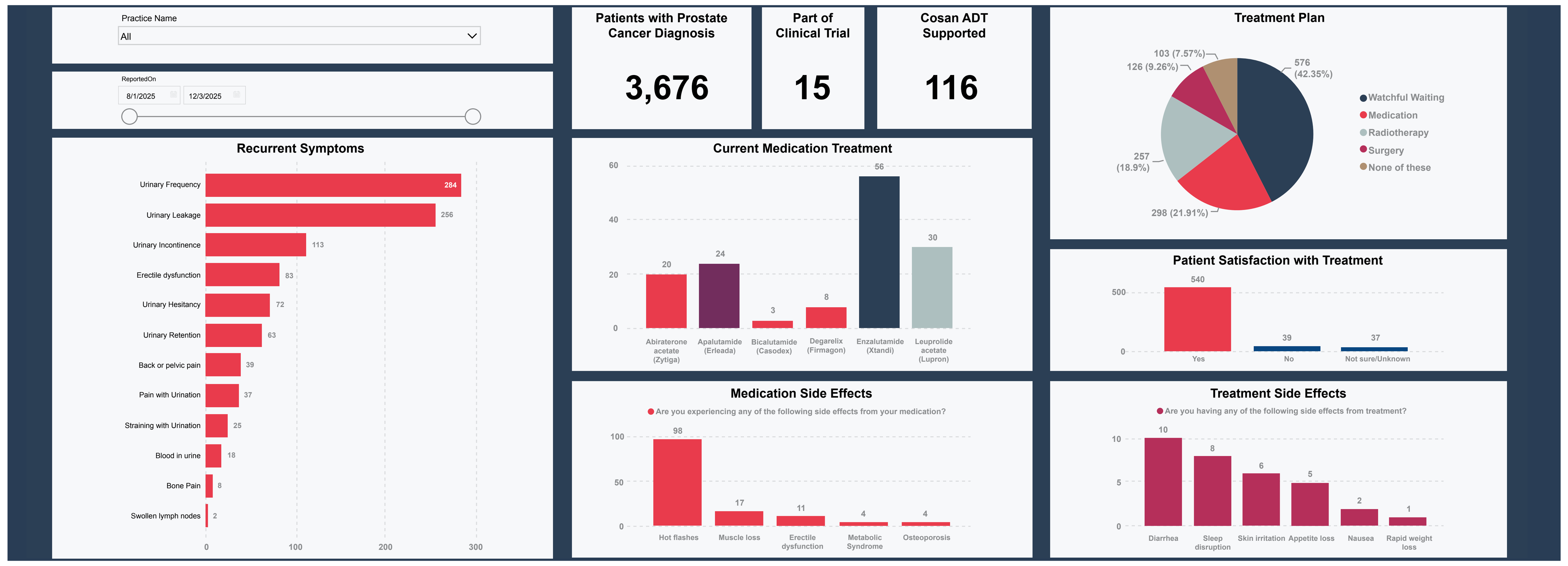
7% of patients reported not taking their prescribed ADT medication

7 patients reported barriers to care

The most frequently documented risks included falls (25), recent hospitalization (19), emergency department visits (14), and urgent care visits (4)

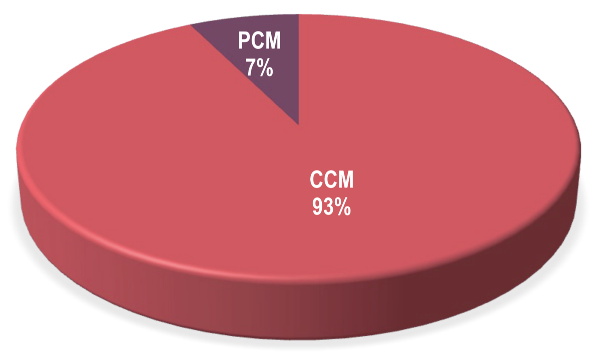
DASHBOARD

An actionable dashboard was developed to track patient outcomes across domains, facilitate communication among providers, and identify care gaps in real time.

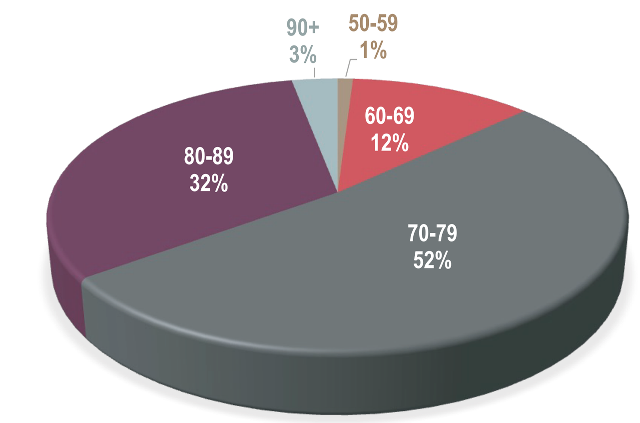

Practice Name
All
ReportedOn
8/1/2025
12/3/2025
Patients with Prostate Cancer Diagnosis
3,676
Part of Clinical Trial
15
Cosan ADT Supported
116
Treatment Plan
576 (42.35%)
298 (21.91%)
257 (18.9%)
126 (9.26%)
103 (7.57%)
Watchful Waiting
Medication
Radiotherapy
Surgery
None of these
Recurrent Symptoms
Urinary Frequency 284
Urinary Leakage 256
Urinary Incontinence 113
Erectile dysfunction 83
Urinary Hesitancy 72
Urinary Retention 63
Back or pelvic pain 39
Pain with Urination 37
Straining with Urination 25
Blood in urine 18
Bone Pain 8
Swollen lymph nodes 2
Current Medication Treatment
Abiraterone acetate (Zytiga) 20
Apalutamide (Erleada) 24
Bicalutamide (Casodex) 3
Degarelix (Firmagon) 8
Enzalutamide (Xtandi) 56
Leuprolide acetate (Lupron) 30
Patient Satisfaction with Treatment
Yes 540
No 39
Not sure/Unknown 37
Medication Side Effects
Are you experiencing any of the following side effects from your medication?
Hot flashes 98
Muscle loss 17
Erectile dysfunction 11
Metabolic Syndrome 4
Osteoporosis 4
Treatment Side Effects
Are you having any of the following side effects from treatment?
Diarrhea 10
Sleep disruption 8
Skin irritation 6
Appetite loss 5
Nausea 2
Rapid weight loss 1

INITIAL BASELINE RESULTS

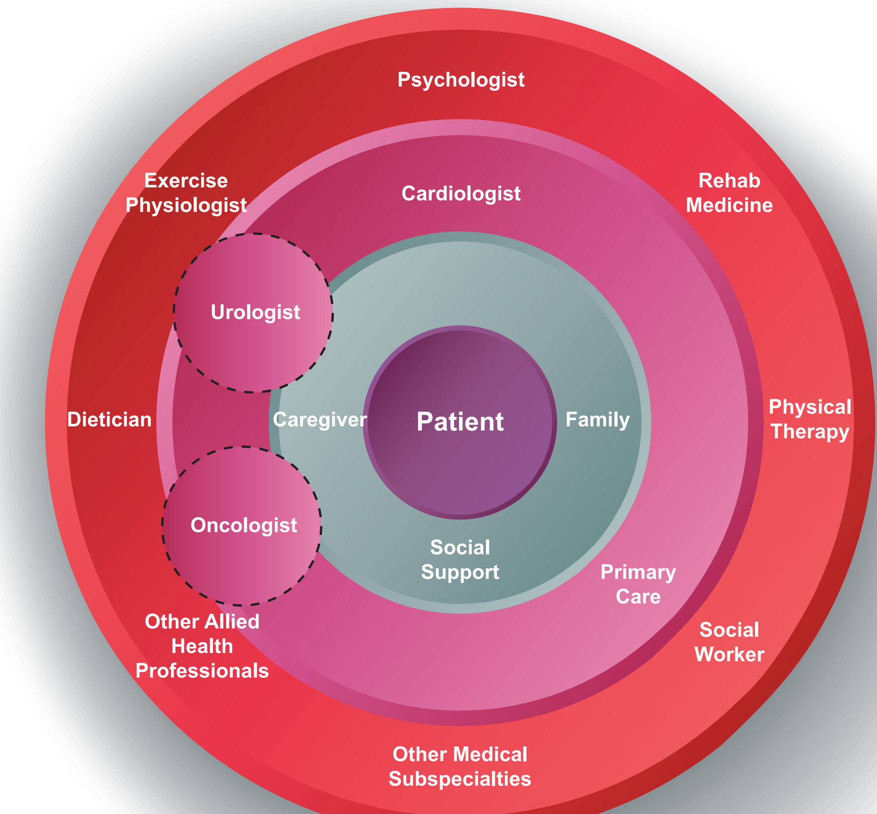
Chronic Care Management (CCM) vs Principal Care Management (PCM)


PCM
7%
CCM
93%

The Age Distribution of Patients


90+
3%
50-59
1%
60-69
12%
70-79
52%
80-89
32%

CHRONIC CARE MANAGEMENT


Psychologist
Exercise Physiologist
Cardiologist
Rehab Medicine
Urologist
Dietician
Caregiver
Patient
Family
Physical Therapy
Oncologist
Social Support
Primary Care
Other Allied Health Professionals
Social Worker
Other Medical Subspecialties

Prescribers should keep in mind the following overarching considerations when deciding on ADT treatment:
- Ensure the patient has a PCP who is aware of the patient's diagnosis and treatment plan, refer patients to a PCP if they do not have one
- Perform baseline assessments for cardiometabolic, bone, and sexual health as well as depression screening and physical activity level assessment
- Intervene when necessary for cardiovascular risk mitigation, including prescribing statins
- Promote inclusion of care partners and trusted support providers in treatment discussions
- Refer patients with uncontrolled risk factors out for additional assessment
- Consider relying on trained clinical staff to provide patient counseling specific to health domains affected

CONCLUSION

Implementation of the PC360 multidisciplinary care model is feasible in routine practice and provides a structured approach to proactively monitor and manage ADT-associated morbidity. Early findings suggest improved care coordination and identification of comorbidity risk factors. Ongoing follow up will evaluate long-term clinical outcomes.

LIMITATIONS AND GRANT SUPPORT

This study is ongoing, and the results presented reflect preliminary data from the initial enrollment period.

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